

# Clinical Summary Document

2025-10-29\_14-09-00

## SUBJECTIVE

Chief Complaint: Patient presents with tick bite discovered behind left knee.

History of Present Illness:

- Patient discovered tick behind left knee in joint flexure area
- Reports local tenderness and redness at bite site
- Outdoor exposure: hiking on Friday and gardening on Saturday
- Exact time of tick attachment unknown
- Tick was removed by patient and brought in for examination
- Tick appeared small and not significantly engorged
- No characteristic erythema migrans (bullseye rash) present
- No systemic symptoms reported

Past Medical History:

- Previous tick bite approximately 10 years ago with erythema migrans, treated with antibiotics
- Hypercholesterolemia, managed with statin medication (specific type unknown)

Medications:

- Unspecified statin for cholesterol management

Allergies:

- No known drug allergies
- No other allergies reported

Social History:

- Lives with husband
- Engages in outdoor activities including hiking and gardening
- No pets reported

## OBJECTIVE

Physical Examination:

- Bite site: Located posterior to left knee
- Local erythema and tenderness present
- No characteristic erythema migrans
- Tick specimen: Small, non-engorged, preserved in container
- Location of exposure: Strafford creek area (known endemic region for Lyme disease)

Vital Signs:

[Not documented in transcript]

## ASSESSMENT

1. Tick bite with potential Lyme disease exposure (Primary)
  - Risk factors: Geographic location in endemic area

- Duration of attachment potentially >72 hours
  - No current evidence of active infection
2. Hypercholesterolemia (Stable, Pre-existing)

## PLAN

1. Prophylactic antibiotic therapy pending physician consultation
2. Submit tick for species identification
3. Consider Lyme disease serology testing
4. Patient education provided regarding:
  - Tick bite prevention
  - Signs/symptoms to monitor
  - Follow-up instructions
5. Recommended continued monitoring for development of:
  - Erythema migrans
  - Constitutional symptoms
  - Joint symptoms

## CODING & BILLING:

E/M Service Level: To be determined based on physician examination

Diagnosis Codes:

- Z20.828 (Contact with and suspected exposure to other viral diseases)
- E78.00 (Pure hypercholesterolemia, unspecified)

## ADMINISTRATIVE:

Authorization Status: Pending

Payer Information: Not provided

Documentation Compliance: Note meets medical necessity criteria

Follow-up: As needed based on symptoms

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